



FAMILY PROSPECTUS

When your family needs *more than hope.*

A 19-bed residential drug rehabilitation centre in Lekki, Lagos. Twelve weeks of clinical, faith-integrated recovery — for families ready to begin.

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A complete walkthrough for prospective families: who House of Refuge serves, the programme, what it costs, and exactly how every step is protected.

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SECTION ONE

When your family needs *more than hope.*

A clinical sanctuary, rooted in faith — for the family making one of the hardest decisions of their lives.



A clinical sanctuary, rooted in faith.

House of Refuge is a 19-bed residential drug rehabilitation centre in Lekki, Lagos — where international clinical standards and faith-integrated recovery are not in tension, but in partnership. Twelve weeks. A clinically-led programme. A son or daughter who comes home whole.

Every resident receives the same clinical standard.

The same Medical Director, the same protocols, the same nursing care. What you pay for is privacy and amenity — never better medicine.

What you can expect, immediately

- An on-site Medical Director, daily, with 24/7 nursing and supervised detox throughout.
- Visiting consultant psychiatrist (2 sessions per week) for specialist consultation.
- Clinical care delivered to WHO and UNODC protocols, with daily multidisciplinary case reviews.
- A complete admission decision in writing — accept, decline, or referral — within five working days of clinical assessment.
- A mandatory pastoral interview with an assigned pastor from the HOR chaplaincy team, attended by patient and family.
- An honest answer about your loved one's fit before any commitment beyond the deposit.

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SECTION TWO

Is House of Refuge right for your loved one?

Recovery is not something that can be done *to* a person — it is something we walk through *with* them. We are honest about who we can and cannot help.



The Willingness Gateway.

The willingness of the person seeking treatment is the single most important factor in long-term recovery. It is the very first thing we assess — before any clinical workup, before any commitment beyond a free conversation.

NON-NEGOTIABLE

Voluntary willingness is the gateway to admission.

Before any clinical assessment, our team holds a structured conversation with your loved one — assessing readiness for change using the URICA framework and motivational interviewing.

If they are not yet willing, we do not force admission. Instead, we offer an Outpatient Engagement Pathway to meet them where they are.

What this means in practice

- **We never simply turn a family away.** Even if residential treatment is not the right fit today, we offer a pathway, written referrals, and continued contact.
- **We talk to your loved one directly.** The willingness conversation is with the person who would be admitted — not with their family on their behalf.
- **Your loved one can change their mind.** A "no" today can become a "yes" later.



Five inclusion criteria.

All five must be met for residential admission. The criteria exist to keep every resident safe.

1 Voluntary Willingness

Your loved one personally consents to treatment and demonstrates readiness for change.

2 SUD Diagnosis

A clinically confirmed substance use disorder, established through ASI or ASSIST screening.

3 Medical Stability

Cleared on the nine pre-admission medical tests and medically safe to undergo supervised detox.

4 Functional Capacity

Able to participate in daily therapy, group sessions, and structured residential life.

5 No Immediate Risk

No active threat of harm to self or others requiring acute psychiatric containment.

The nine pre-admission medical tests

HIV · Hepatitis A & B · Urine Drug Screen · Full Blood Count · Liver Function Tests · Malaria · Widal · Chest X-Ray · Blood Glucose.



What we cannot safely treat in residence.

We are honest with families about our limits. The presentations below require an acute psychiatric or specialist setting — not a residential rehabilitation programme.

Active psychosis

Severe suicidal or homicidal risk

Drug-induced psychosis

Active antipsychotic medication need

Severe cognitive impairment

Active legal detention

Dependent children requiring co-habitation

How we handle a "no"

- The clinical decision is delivered to the family **in writing**, with the reasoning behind it.
- Every declined applicant receives a **written referral** to a more appropriate care setting.
- The 600,000 portion of the deposit is **refunded in full** within 14 days. The 400,000 assessment fee is retained — the clinical work was done.

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SECTION THREE

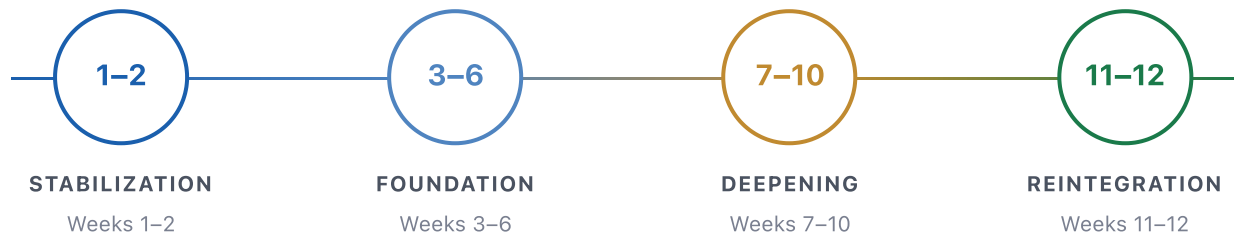
Twelve weeks.
Four phases.
One clinically-led pathway.

Each phase has defined goals, weekly clinical reviews, and clear family touchpoints.



The twelve weeks at a glance.

The HOR programme is delivered across four clinical phases over twelve weeks.



What you can expect each week

- **Daily group therapy** — CBT, 12-step, and faith-based work, delivered by qualified clinicians.
- **Weekly individual counselling** — daily on the Signature tier, three times weekly on Premium.
- **Weekly multidisciplinary case review** — your loved one's progress is examined every week.
- **Faith and chaplaincy programme** — daily, woven into the daily schedule.
- **Family touchpoints** — at least one every fortnight, more often by clinical request.



Phases 1 & 2 — Stabilization and Foundation.

The first six weeks: settling the body, stabilising the mind, and laying the therapeutic foundation.

PHASE ONE

Medical Stabilization

Weeks 1–2

- Supervised medical detox with 24/7 nursing
- Vitals, nutrition, and sleep stabilization
- Full programme orientation
- Initial spiritual care + chaplaincy

FAMILY TOUCHPOINT

Welcome call within 72 hours. Orientation pack delivered. Visitation begins from Week 2.

PHASE TWO

Therapeutic Foundation

Weeks 3–6

- Group CBT and relapse-prevention work
- Weekly 1:1 counselling (daily on Signature)
- Psychoeducation, life skills, fitness
- Week 4 multidisciplinary treatment review

FAMILY TOUCHPOINT

First family therapy session by Week 4. Sunday visitation each week.



Phases 3 & 4 — Deepening and Reintegration.

The second six weeks: trauma processing, family-systems work, and a structured exit into a real life.

PHASE THREE

Deepening & Skills

Weeks 7–10

- Trauma processing and family-systems work
- Vocational assessment and planning
- Personal Relapse Prevention Plan (PRPP)
- Week 8 progress review with the MDT

FAMILY TOUCHPOINT

Family-systems sessions deepen. 24-hour and 48-hour passes become available based on conduct.

PHASE FOUR

Reintegration

Weeks 11–12

- Pre-discharge clinical assessment
- Aftercare and reintegration plan
- Graduation ceremony
- Alumni Programme enrolment

FAMILY TOUCHPOINT

Pre-discharge family meeting. Graduation ceremony. Year-one aftercare schedule confirmed.

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SECTION FOUR

Programme fees, admission flow, and exactly how your money is held.

A clinical decision and a financial decision happen together.



Two paying tiers. One clinical standard.

Same Medical Director, same protocols, same nursing, same WHO/UNODC framework. What you pay for is privacy, amenity, and the rhythm of one-to-one contact.

SIGNATURE

3 beds · always limited

NGN 4,500,000 /month

3-month total: NGN 13,500,000

- ✓ Private en-suite room — single occupancy
- ✓ Daily 1:1 therapy with a clinical psychologist
- ✓ 4 family sessions per month
- ✓ Personal chaplain — weekly
- ✓ Concierge admissions with private transport
- ✓ Signature menu — broader, with premium options
- ✓ Vocational planning + aftercare

PREMIUM

16 beds

NGN 2,800,000 /month

3-month total: NGN 8,400,000

- ✓ En-suite room — 2-bed shared
- ✓ 1:1 counselling — 3 sessions per week
- ✓ 2 family sessions per month
- ✓ Group chaplaincy — daily
- ✓ Concierge admissions + dedicated keyworker
- ✓ Premium menu (Signature menu add-on available)
- ✓ Reintegration plan + aftercare

A NOTE ON FINANCIAL ASSISTANCE

Need-based admission is available to families experiencing genuine financial hardship. Applications reviewed independently by the **Freedom Foundation Financial Assistance Committee**. The clinical standard does not change. Placements operate on a **separate waitlist with unpredictable wait times** — fully donor-funded, capacity extremely limited. Apply at houseofrefugeng.org/assistance. Sponsorship inquiries: **0911 277 7600**.



The admission flow — six stages.

Six stages between your first call and your loved one's first day.

- 1

Self-assessment
Free · About 30 minutes online
Structured intake form: substance history, medical history, family context, willingness for treatment.
- 2

Deposit
NGN 1,000,000 · After self-assessment review
Split: 400k assessment fee + 600k held toward Month 1.
- 3

Clinical assessment
Within 7–14 days of deposit
Consultant psychiatrist + 9 pre-admission lab tests + MDT review. Decision in writing within 5 working days.
- 4

Pastoral interview
Assigned chaplaincy pastor · Patient and family · Booked by admissions
A **mandatory** discernment conversation with an assigned pastor from the HOR chaplaincy team. Patient and family attend together.
- 5

Active waitlist (if needed)
Up to 6 months · Monthly check-in · Renewable
19 beds in total. Your 600k is held — reserving your place. Withdraw any time for full 600k refund.
- 6

Admission
Bed offered, family confirms, admission scheduled
600k applied to Month 1. Balance settled before admission day. Phase One — Medical Stabilization — begins.



The 1M deposit, plain.

The admission deposit is NGN 1,000,000. It is split into two parts.

DEPOSIT BREAKDOWN · ITEMISED

Where every Naira goes.

NGN 400,000

Clinical assessment fee

- Psychiatrist consultation (structured interview)
- 9 pre-admission lab tests
- MDT case review + written decision
- Referral to appropriate setting if declined

Consumed once the psychiatrist begins your file.

NGN 600,000

Held toward Month 1

- Reserves your place on the active waitlist
- Applied directly to Month 1 fees on admission
- Refundable in full if you withdraw or we decline
- Refundable if no bed opens within the hold period

Held — never consumed unless you admit.



Refund policy in plain language.

We refund every Naira we have not earned.

If...	Then...
Within 48 hours of paying the deposit, you change your mind	Full NGN 1,000,000 refunded
You withdraw between 48 hours and the psychiatrist consult	600k refunded ; 400k retained
Our psychiatrist clinically declines admission	600k refunded + written referral; 400k retained
You withdraw more than 14 days before admission day	600k refunded
You withdraw within 14 days of admission day	50% of 600k refunded; 50% retained
We cannot offer a bed within your active waitlist hold	600k refunded ; 400k retained
You withdraw voluntarily during the waitlist hold	600k refunded in full
You decline a slot offer once made	600k refunded
Within 72 hours of admission, your loved one withdraws	80% of total Month 1 paid; 400k not refundable
Withdrawal after 72 hours of admission	No refund (no exceptions)
Medical discharge after Month 1 (clinical reasons)	Months 2+ refunded pro-rata



How the waitlist actually works.

A 19-bed facility means there will be times when no bed is immediately available. The active waitlist exists to make the wait fair, transparent, and reversible.

- **Active hold lasts 6 months.** Renewable on confirmation if you wish to keep waiting.
- **Monthly check-in.** Admissions contacts you each month. Two missed contacts releases the hold and refunds 600k.
- **Withdraw whenever you need to.** 600k refunded in full.
- **One decline is a clean exit.** 600k refunded. No haggling.
- **Re-assessment after 6 months.** NGN 100,000.

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SECTION FIVE

The rhythm of recovery.

Plate by plate, hour by hour. Device-free, by design.



Meals as part of the medicine.

Detox is exhausting work for the body. Sleep, hydration, blood sugar, and basic nutrition all suffer through addiction — and recovery is partly the slow business of putting them back together.

Our catering partner **CookedIndoors** prepares every meal fresh, with menus shaped alongside the clinical team. The food is warm, varied, and deliberately familiar.

NUTRITION MEETS CLINICAL CARE

Every resident's plan is reviewed by the clinical team at intake — and adjusted through the programme as recovery progresses. **Halal, vegetarian, diabetic, hypertensive, allergy-led** — every dietary need is accommodated, without families having to negotiate it.



A device-free environment.

House of Refuge is, by design, a programme separated from the constant pull of digital life.

NO PHONES · NO COMPUTERS · NO PERSONAL DEVICES

A focused, distraction-free environment for recovery.

No mobile phones, computers, or personal digital devices are permitted at the facility for the duration of the residential programme — a deliberate part of creating a focused, distraction-free environment for recovery. Staying connected with family is coordinated through the programme's **structured family sessions**.

Why this matters clinically

- Constant connectivity is one of the most common relapse vectors — old contacts, old triggers, old habits, one tap away.
- The recovery work happens in present, embodied attention. Devices erode that.
- Family relationships are restored through deliberate conversations, not passive contact.

The daily rhythm — meals as anchor points

8:00 AM	Breakfast, after morning prayer
10:30 AM	Mid-morning snack — shared
12:30 PM	Lunch
3:30 PM	Afternoon snack — shared
6:00 PM	Dinner, before evening chapel



WHEN YOU ARE READY

Begin admission.

An admissions counsellor will respond within one working day, in confidence. There is no obligation, and no commitment beyond the conversation, until the self-assessment is reviewed and you choose to continue.

PHONE

0911 277 7600

Admissions · 9 AM – 6 PM, Mon–Sat

ONLINE

houseofrefugeng.org

Self-assessment form · ~30 minutes

Location

Lekki, Lagos ·
Nigeria

Capacity

19 residential
beds

Programme

12 weeks · Extendable to 6
months

Confidential

A Freedom Foundation
Initiative